

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
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Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

June 29, 2022

MEMORANDUM FOR: (b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
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ICE Health Service Corps

THROUGH: (b)(6),(b)(7)(C),(b)(7)(F) MD (b)(6),(b)(7)(C),(b)(7)(F)
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FROM: CDR (b)(6),(b)(7)(C),(b)(7)(F), RN (b)(6),(b)(7)(C),(b)(7)(F)
Senior Investigator
ICE Health Service Corps

SUBJECT: Mortality Review – Report of Findings
Carlos Ernesto ESCOBAR Mejia, (b)(6),(b)(7)(C)

Executive Summary:

On May 6, 2020, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE detainee Carlos Ernesto ESCOBAR Mejia, (b)(6),(b)(7)(C). Mr. ESCOBAR Mejia, a 57-year-old El Salvadoran male, was in ICE custody from January 10 to May 6, 2020, and assigned to Otay Mesa Detention Center (OMDC) in San Diego, California (CA), on the date of his death. An autopsy ruled Mr. ESCOBAR Mejia's death resulted from acute respiratory failure, due to pneumonia and coronavirus disease 2019 [(COVID-19); infectious disease caused by the severe acute respiratory syndrome coronavirus (SARS-CoV-2)].

Mortality Review – Carlos Ernesto ESCOBAR Mejia, (b)(6),(b)(7)(C)

Page 2 of 9

Prior to ICE custody, on January 3, 2020, U.S. Border Patrol (USBP) arrested Mr. ESCOBAR Mejia near Campo, CA, on suspicion of alien smuggling.

On January 10, 2020, USBP transferred Mr. ESCOBAR Mejia to ICE Enforcement and Removal Operations (ERO) custody in San Diego, CA, and ERO housed Mr. ESCOBAR Mejia at OMDC.

During Mr. ESCOBAR Mejia's detention, OMDC provided him medical care for hypertension [(HTN); high blood pressure], type II insulin dependent diabetes mellitus (T2DM), Vitamin D deficiency, hypercholesterolemia (elevated cholesterol level), and surgical amputation of his right foot, occurring three months prior to his detention, due to T2DM complications.

On April 21, 2020, an advanced practice provider (APP) documented Mr. ESCOBAR Mejia's complaints of chills, cough, sore throat, and shortness of breath (SOB) for several days, diagnosed a viral infection, and ordered a SARS-CoV-2, nucleic acid amplification (NAA) (COVID-19 test).

On April 24, 2020, OMDC received Mr. ESCOBAR Mejia's COVID-19 test, showing a positive result, and reassigned him to a designated housing unit for patients with positive COVID-19 results. A registered nurse (RN) assessed Mr. ESCOBAR Mejia, noted a temperature (T) of 98.7 degrees Fahrenheit [(F); normal range 97.8 – 99.1 F]; pulse (P) of 106 beats per minute [(bpm) normal range 60 – 100 bpm]; respiratory rate (R) of 22 breaths/minute [(bpm); normal range 12 – 18 bpm]; blood pressure (BP) of 130/94 millimeters of mercury [(mmHG); normal range 90/60 – 120/80 mmHG]; and oxygen saturation [(SpO2); level of oxygen carried by blood cells through the body] of 90 percent [(%); normal range 95 – 100%].

On examination, the RN documented Mr. ESCOBAR Mejia's complaint of sore throat, rated three out of ten [(3/10); pain scale 0 to 10], fatigue, assistance needed with ambulation, SOB with exertion, skin warm and dry to touch, positive non-productive cough, and his SpO2 level did not improve with activity or deep breaths. The RN notified the APP on-site, and obtained orders to transfer Mr. ESCOBAR Mejia, via emergency medical services (EMS), to the Paradise Valley Hospital (PVH) emergency department (ED), in National City, CA, for further evaluation. The RN assisted Mr. ESCOBAR into a wheelchair, administered oxygen at six (6) liters (L)/minute via face mask, and his SpO2 level improved from 90% to 96%. Upon arrival to the facility, EMS personnel assumed care. Prior to departure, the RN documented Mr. ESCOBAR Mejia's disposition as calm, cooperative, conversing with staff, alert, and oriented.

PVH admitted Mr. ESCOBAR Mejia to medical services and diagnosed him with T2DM, HTN, COVID-19 infection, and pneumonia.

On April 27, 2020, PVH transferred Mr. ESCOBAR Mejia to the intensive care unit due to becoming increasingly SOB, increased respiratory rate (33 bpm), and decreased SpO2 level of 87%.

Mortality Review – Carlos Ernesto ESCOBAR Mejia, (b)(6),(b)(7)(C)

Page 3 of 9

From April 27 to May 6, 2020, Mr. ESCOBAR Mejia's condition continued to deteriorate. On May 6, 2020, Mr. ESCOBAR Mejia experienced cardiac arrest (sudden unexpected cessation of heart function, breathing and consciousness) and a medical doctor (MD) pronounced him deceased at 2:15 a.m.

Mortality Finding:

As a result of Mr. ESCOBAR Mejia's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues that require further study.

A mortality review committee (MRC) determined that Mr. ESCOBAR's medical care at OMDC was provided within the safe limits of practice.

The MRC convened on September 21, 2021, and consisted of the following IHSC members: Captain (b)(6),(b)(7)(C),(b)(7)(F) Western Regional Nurse Manager; Dr. (b)(6),(b)(7)(C),(b)(7)(F) Central Regional Medical Director; and Ms. (b)(6),(b)(7)(C),(b)(7)(F) APP.

The following summary of events was based on: 1) Mr. ESCOBAR Mejia's OMDC medical records; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. ESCOBAR Mejia's OMDC detention file; 5) a remote review and staff interviews conducted by factfinder, Commander (b)(6),(b)(7)(C),(b)(7)(F) on July 2-15, 2020; and 6) applicable OMDC policies and procedures, and ICE Detention Standards.

Detailed Medical Review

On January 10, 2020, an RN completed Mr. ESCOBAR's pre-screening and intake screening, and noted a self-reported history of HTN for twelve years, T2DM for five years, Vitamin D deficiency for one year, hypercholesterolemia for three years, and surgical amputation of his right foot, three months prior to detention, due to T2DM complications. Mr. ESCOBAR Mejia's medications were: Novolin Regular and Lantus insulins (anti-diabetic medications); lisinopril and amlodipine (anti-hypertensive medications); and Vitamin D3 supplementation. Additionally, Mr. ESCOBAR Mejia reported a prosthetics clinic evaluated him for a right foot prosthesis (artificial foot) prior to his detention. The RN issued Mr. ESCOBAR a wheelchair to aid his mobility and scheduled a complex physical exam (PE-C) for January 11, 2020.

On January 11, 2020, an APP completed Mr. ESCOBAR Mejia's PE-C, diagnosed the following conditions: HTN, T2DM, hyperlipidemia (elevated lipids in the blood); and prescribed: amlodipine besylate, 5 milligrams (mg), one tablet, by mouth (PO), every morning (qAM), keep

Mortality Review – Carlos Ernesto ESCOBAR Mejia, (b)(6),(b)(7)(C)

Page 4 of 9

on person [(KOP; self-administered medication)]; lisinopril, 5 mg, one tablet, PO, qAM, KOP; metformin hydrochloride (anti-diabetic medication), 1000 mg, one tablet, with meals, PO, two times daily (BID), KOP; glargine insulin, 100 units per milliliter (ml), 14 units, subcutaneous (between the skin and muscle) injection, every evening (qPM); glucose (treatment for low blood sugar level) tablets, four grams, PO, BID, as needed (PRN), KOP; and cholecalciferol (Vitamin D supplement), 2000 units, one tablet, PO, qAM, KOP. In addition, the APP ordered the following laboratory tests: prostate specific antigen [(PSA); measurement of prostate-specific antigen in blood], lipid panel (measurement of lipid levels), comprehensive metabolic panel (CMP), complete blood count (CBC), urinary analysis (UA), hemoglobin A1c [(HgbA1c); determines level of diabetes control over three months], and Vitamin D levels. The APP scheduled a follow-up visit in two weeks.

On January 13, 2020, an RN obtained a blood sample from Mr. ESCOBAR Mejia for the lab tests ordered on January 11 and returned Mr. ESCOBAR Mejia to his housing unit.

On January 15, 2020, a dentist evaluated Mr. ESCOBAR Mejia for complaint of dental pain rated 7/10. His vital signs (VS) were normal except for a BP reading of 180/92 mmHg and 183/91 mmHg. The dentist extracted tooth #15 without complication and treated his post-op pain with ibuprofen (oral analgesic). The dentist reported Mr. ESCOBAR Mejia's elevated BP to an APP, who scheduled every-other-day BP checks for one week, and notified Mr. ESCOBAR Mejia's primary care (PC) APP via a telephone encounter in the IHSC electronic medical record. Mr. ESCOBAR Mejia had a previously scheduled follow-up appointment with his PC APP on January 23, 2020; however, OMDC medical staff did not address Mr. ESCOBAR Mejia's elevated BP reading on January 15.

On January 16, 2020, a licensed vocational nurse (LVN) documented Mr. ESCOBAR Mejia's BP of 146/74 mmHg.

On January 17, 2020, an RN evaluated Mr. ESCOBAR Mejia related to his request for a prosthetic foot. The RN documented his BP reading of 155/85 mmHg and submitted his request to an APP.

On January 18, 2020, an RN documented Mr. ESCOBAR Mejia's BP reading of 156/98 mmHg.

On January 19, 2020, an LVN documented Mr. ESCOBAR Mejia's BP reading of 148/92 mmHg.

On January 20, 2020, an LVN documented Mr. ESCOBAR Mejia's BP reading of 150/101 mmHg.

On January 23, 2020, an APP completed Mr. ESCOBAR Mejia's follow-up evaluation. His VS were normal except a BP reading of 156/90 mmHg. The APP reviewed the lab results with Mr.

Mortality Review – Carlos Ernesto ESCOBAR Mejia, (b)(6),(b)(7)(C)

Page 5 of 9

ESCOBAR Mejia. The abnormal findings were albumin 2.3 (normal 3.5-5.5), protein 4.6 (normal 6.0-8.5), calcium 8.1 (normal 8.7-10.2), glucose 197 (normal 65-99), alkaline phosphatase 123 (normal 39-117), hemoglobin and hematocrit 10.6/33.6 (normal 13.0-17.7 / 37.5-51.0), red blood cells (RBC) 3.86 (normal 4.14-5.80), hemoglobin A1c 9.1 (normal 4.8-5.6), and vitamin D 9.6 (normal 30.0-100.0). The APP increased the lisinopril dosage to 10 mg PO qAM, ordered regular insulin per sliding scale (varies insulin dosage based on blood glucose level), increased the Lantus insulin dosage to 16 units qPM, ordered referral to orthotics clinic for right foot prosthesis, and scheduled a follow-up in two weeks.

On January 30, 2020, an APP completed Mr. ESCOBAR Mejia's follow-up evaluation. His VS were normal except a BP reading of 165/92 mmHg. The APP increased his lisinopril to 20 mg PO qAM and planned to follow up in one week.

On February 3, 2020, an RN documented Mr. ESCOBAR Mejia's BP reading of 149/178 mmHg.

On February 4, 2020, an RN documented Mr. ESCOBAR Mejia's BP reading of 167/186 mmHg.

On February 5, 2020, an RN documented Mr. ESCOBAR Mejia's BP reading of 148/78 mmHg.

On February 6, 2020, an RN documented Mr. ESCOBAR Mejia's BP reading of 192/106 mmHg. Mr. ESCOBAR Mejia reported to the RN that he had forgotten to take his medication, and the RN instructed him to take the medications now; and 30 minutes later, his repeat BP reading was 144/85 mmHg.

On February 7, 2020, an RN documented Mr. ESCOBAR Mejia's BP reading of 149/89 mmHg.

On February 10, 2020, an APP completed Mr. ESCOBAR Mejia's follow-up evaluation. His VS were normal except a BP reading of 134/92 mmHg, exam normal, no new orders, and the APP scheduled a follow-up in four weeks.

On February 17, 2020, an RN documented Mr. ESCOBAR Mejia's BP reading of 150/87 mmHg.

On February 18, 2020, an RN documented Mr. ESCOBAR Mejia's return from a Hangers Prosthetic Clinic appointment. His BP reading was 163/88 mmHg.

On February 19, 2020, an RN documented Mr. ESCOBAR Mejia's BP reading as 162/90 mmHg.

Mortality Review – Carlos Ernesto ESCOBAR Mejia, (b)(6),(b)(7)(C)

Page 6 of 9

On February 24, 2020, an APP completed Mr. ESCOBAR Mejia's follow-up evaluation. His VS were normal except a pulse rate of 58 bpm. The exam was normal. The APP increased the lisinopril dosage to 30 mg PO qAM, educated Mr. ESCOBAR Mejia on the use of the shrinker sock (elastic sock that is used to control swelling, promote healing, and assist in shaping an amputated leg) in preparation for prosthetic fitting, and scheduled a follow-up evaluation in three weeks.

On March 18, 2020, an APP completed Mr. ESCOBAR Mejia's follow-up evaluation. His VS were normal except for a BP reading of 146/86 mmHg. The exam was normal. The APP increased the lisinopril dosage to 40mg PO qAM and scheduled a follow-up in two weeks.

On April 1, 2020, an APP completed Mr. ESCOBAR Mejia's follow-up evaluation. His VS were normal except for a BP reading of 157/81 mmHg. The exam was normal. The APP changed the topical terbinafine (anti-fungal) to oral terbinafine, 250mg, PO qAM for 90 days, and scheduled a follow-up in four weeks.

On April 2, 2020, an RN evaluated Mr. ESCOBAR Mejia for toenail trimming. His VS were normal except a BP reading of 145/84 mmHg. The exam was normal. The RN permitted Mr. ESCOBAR Mejia to trim his own toenails, resulting in him cutting one nail too close causing the toe to bleed. An APP examined the toe wound and ordered wound care.

On April 3, 2020, an APP completed Mr. ESCOBAR Mejia's follow-up evaluation. His VS and toe exam were normal. During the exam, Mr. ESCOBAR Mejia complained of indigestion, and the APP prescribed calcium carbonate (dietary supplement) chewable tablets 500 mg, one tablet three times a day for thirty days.

On April 7, 2020, an RN documented Mr. ESCOBAR Mejia returned from a Hangers Prosthetic Clinic appointment. His BP reading was 160/80 mmHg. The RN scheduled an appointment with an APP for April 8, 2020.

On April 8, 2020, an APP completed Mr. ESCOBAR Mejia's follow-up evaluation. His VS were normal. The Hanger Prosthetic Clinic notes were not available, so the APP scheduled a follow-up in two weeks.

On April 14, 2020, an APP reviewed the Hanger Prosthetic Clinic notes and ordered a follow-up outside appointment in a few weeks.

On April 19, 2020, an APP evaluated Mr. ESCOBAR Mejia for recurring low blood glucose levels in the mornings, ranging from 50-74 milligrams per deciliter (mg/dL). His VS were normal except for a BP reading of 164/91 mmHg. The exam was normal. The APP decreased the glargine dosage to 15 units every evening and ordered a UA, CMP, and HgbA1c for April 28, 2020.

Mortality Review – Carlos Ernesto ESCOBAR Mejia, (b)(6),(b)(7)(C)

Page 7 of 9

On April 21, 2020, an APP evaluated Mr. ESCOBAR Mejia for complaints of chills, cough, sore throat, and SOB for several days. The APP did not identify abnormal findings, diagnosed a viral infection, and ordered a SARS-CoV-2, NAA.

On April 23, 2020, an RN informed an APP of Mr. ESCOBAR Mejia's glucose level of 41 mg/dL and gave him a sandwich and crackers, but the RN did not recheck his glucose level.

On April 24, 2020, Mr. ESCOBAR Mejia's COVID-19 test result was positive and OMDC reassigned him to a designated housing unit for patients with positive COVID-19. An RN assessed Mr. ESCOBAR Mejia noting the following VS: T of 98.7 degrees (F), P of 106 bpm, R of 22 breaths/minute, BP of 130/94 mmHG, and SpO2 of 90 %. On examination, the RN documented Mr. ESCOBAR Mejia's complaint of sore throat, rated 3/10, fatigue, assistance needed with ambulation, SOB with exertion, skin warm and dry to touch, positive non-productive cough, and his SpO2 level did not improve with activity or deep breaths. The RN notified the APP on duty and obtained orders to transfer Mr. ESCOBAR Mejia, via EMS, to PVH ED for evaluation. The RN assisted Mr. ESCOBAR Mejia in a wheelchair, administered oxygen at 6 L/m via face mask, and his SpO2 level improved from 90% to 96%. Upon arrival to the facility, EMS personnel assumed care. Prior to departure, Mr. ESCOBAR Mejia's disposition as calm, cooperative, conversing with staff, alert, and oriented.

- PVH admitted Mr. ESCOBAR Mejia as an in-patient with diagnoses of T2DM, HTN, COVID-19 infection and pneumonia.

From April 24 to April 26, 2020, PVH assigned Mr. ESCOBAR Mejia to a regular room and initiated treatment for each of his admitting diagnoses with no appreciable changes in his condition.

On April 27, 2020, PVH transferred Mr. ESCOBAR Mejia to the intensive care unit due to becoming increasingly hypoxic, increased respiratory rate (33 bpm), and decreased SpO2 level of 87%.

On April 28, 2020, PVH reported Mr. ESCOBAR Mejia experienced hypotension (low BP), and his respiratory rate averaged 26 bpm. As a result, PVH sedated Mr. ESCOBAR Mejia, inserted two central venous catheters, and initiated on a mechanical ventilator and hemodialysis (mechanical filtering of waste products from the blood).

On April 29, 2020, PVH reported Mr. ESCOABAR Mejia developed a pneumothorax (collapsed lung), and they inserted a chest tube to aid breathing. Although PVH aggressively treated his conditions, he continued to worsen, and two of his physicians discussed changing his medical status to do not resuscitate (DNR).

Mortality Review – Carlos Ernesto ESCOBAR Mejia, (b)(6),(b)(7)(C)

Page 8 of 9

On April 30, 2020, PVH changed Mr. ESCOBAR Mejia's status to DNR and continued his current treatments. ICE ERO notified Mr. ESCOBAR Mejia's family of his status.

On May 1, 2020, PVH reported to OMDC that Mr. ESCOBAR Mejia's condition and prognosis remained poor.

On May 2, 2020, PVH decreased Mr. ESCOBAR Mejia's sedation to assess his cognition; however, he remained unresponsive. Although Mr. ESCOBAR Mejia's prognosis remained poor, PVH inserted a feeding tube to provide nutritional supplementation.

On May 3, 2020, PVH reported to OMDC that Mr. ESCOBAR Mejia's condition continued to deteriorate. PVH decreased his sedation and observed a slight, non-purposeful movement.

From May 4 to May 5, 2020, PVH reported to OMDC that Mr. ESCOBAR's condition was unchanged.

On May 6, 2020, PVH's attending physician pronounced Mr. ESCOBAR Mejia dead at 2:15 a.m., PST.

Detention Standards used for this review: ICE Performance-Based National Detention Standards (PBNDS), 2011 (revised December 2016), and the National Commission on Correctional Health Care (NCCHC) Standards, 2018. The following are identified program strengths, weaknesses, and recommendations found during this review.

Strengths

The strength identified during this investigation was the rapid detection of subtle physical changes in Mr. ESCOBAR Mejia's condition, resulting in immediate transfer to a local hospital.

Weaknesses

1. Communication and language services.

- EARM records and the intake screening documentation noted Mr. ESCOBAR Mejia was Spanish speaking. OMDC medical staff did not consistently utilize interpreter services when evaluating Mr. ESCOBAR Mejia.

Applicable standards of care for this finding:

- PBNDS 2011: 4.3, Medical Care; section V. E. *Translation and Language Access for Detainees with Limited English Proficiency*.
- NCCHC 2018, J-E-01, Information on Health Services

Mortality Review – Carlos Ernesto ESCOBAR Mejia, (b)(6),(b)(7)(C)

Page 9 of 9

2. Fraihat classification.

- On arrival to OMDC, ICE classified Mr. ESCOBAR Mejia as Fraihat (ICE civil rights class action lawsuit) low risk. Per ICE ERO COVID-19 Pandemic Response Requirements (PRR), OMDC should have classified Mr. ESCOBAR Mejia as high risk related to HTN and T2DM.

Applicable standards of care for this finding:

- ERO COVID-19 Pandemic Response Requirements Version 5.0.

3. Care.

- On April 23, 2020, an RN reported Mr. ESCOBAR Mejia's blood glucose of 41 mg/dL and completed the APP's orders, but the RN did not recheck Mr. ESCOBAR Mejia's blood glucose.
- The APP was not in compliance with the IHSC Clinical Practice Guidelines treatment protocols for diabetes and HTN.
- OMDC did not offer Mr. ESCOBAR a flu vaccination.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section II Expected Outcomes, item 8.
- NCCHC 2018: J-F-01, Patients with Chronic Disease and other Special Needs.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director of Health Care Compliance (DAD-HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, OMDC's administrator and health authority, for review and any action(s) deemed appropriate.
- The respective IHSC HCC Unit and ICE will ensure the corrective action plan, if applicable, is implemented and sustained.

End of report.